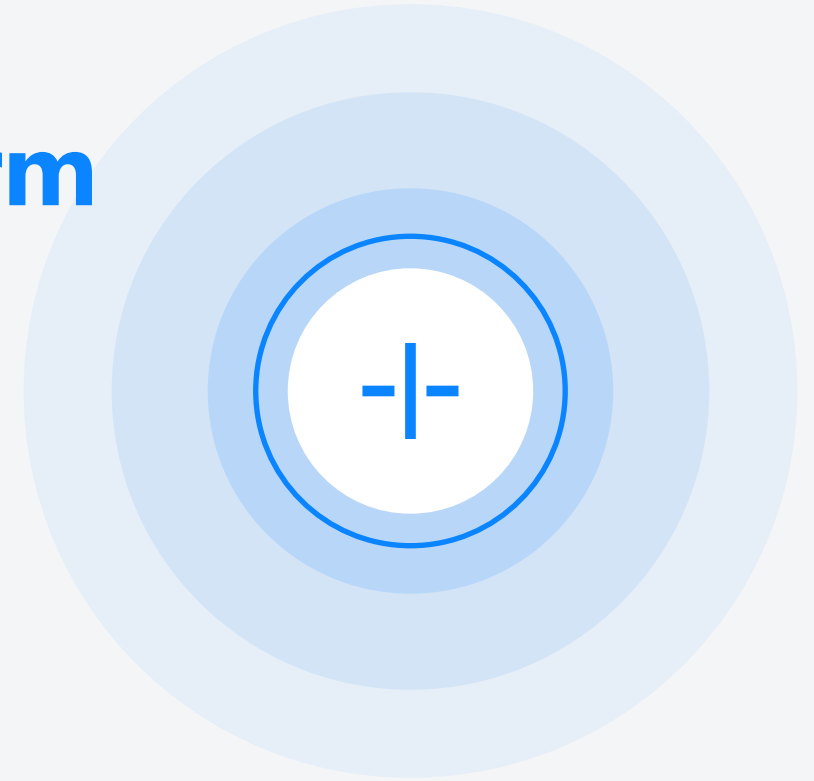


From health prototype to continuous care platform

A practical proposal for patient identity, a responsive doctor portal, consent-driven records, and a safe path to real telehealth.



[INTERACTIVE PDF](#)

JULY 2026

Prepared for the Nirog product and engineering team

THE CONVERSATION

One pitch. One decision.

Use pages 03-10 to align on the product, then complete the interactive decision pages together.

03

Starting point

What the current Expo prototype proves

05

Two surfaces

Patient mobile + doctor web

07

Doctor workspace

Responsive clinical operations

09

Safety baseline

Security, privacy and clinical controls

11

Delivery roadmap

Build foundations before scale

13

Action plan

Assign owners and surface risks

04

North star

The connected care loop

06

Patient journey

Low-friction identity and family care

08

Trust architecture

Identity, consent, data and ARIA

10

Rural advantage

What makes Nirog meaningfully different

12

Team decision

Vote, decide and document

14

Evidence

Repository and standards references

WHAT WE ALREADY HAVE

The prototype proves the care journey

The opportunity is to replace isolated demos with identity, persistence, consent and accountable clinical workflows.

5**patient modules**

ARIA, doctors, shop, locker, home

1**AI intake service**

Bedrock-backed ARIA + handover

0**production identities**

patient and doctor data are simulated

1**missing foundation**

trusted shared backend

CURRENT STATE -> PRODUCT STATE

Today

- Hard-coded patient and family profiles
- Local handover storage
- Demo calls and prescriptions
- ARIA without app user context

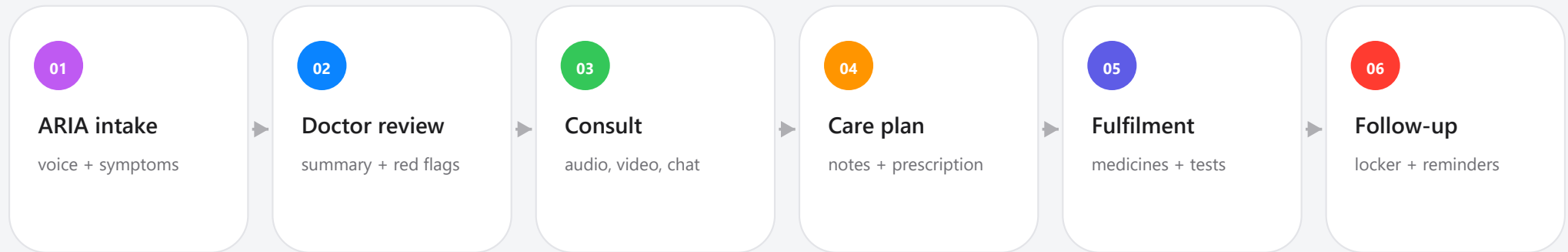
Next

- Verified accounts and care relationships
- Server-side longitudinal health record
- Real encounters and clinician sign-off
- Authenticated, consent-aware ARIA

THE PRODUCT IDEA

Make every interaction part of one care episode

Login is valuable only when it creates continuity across intake, clinical care, treatment and follow-up.



NORTH STAR METRIC

Resolved care episodes

not logins, screens or AI messages

PRODUCT SHAPE

One platform. Two purpose-built experiences.

Share identity, records and API contracts. Do not force patient and clinician work into the same interface.

EXISTING EXPO APP

Patient mobile

- Voice-first and low-bandwidth
- Family and caregiver profiles
- Bookings, records and follow-up
- Guest access for emergency essentials

Shared identity + consent + records + clinical events

NEW RESPONSIVE PORTAL

Doctor web

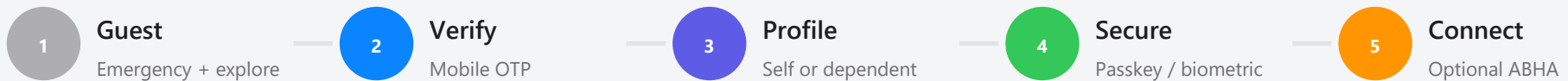
- Queue and appointment operations
- Patient chart + ARIA handover
- Consultation notes and prescribing
- Desktop, tablet and mobile browser

Shared identity + consent + records + clinical events

PATIENT EXPERIENCE

Low-friction login without blocking care

Use identity to protect and preserve health information, while keeping emergency access and discovery open.



● Account is not a patient

One authenticated person can manage self, child, parent or dependent profiles. Every relationship has an explicit scope.

● ABHA is a connection

Keep ABHA optional and external. Never use phone, email or ABHA as the internal database key.

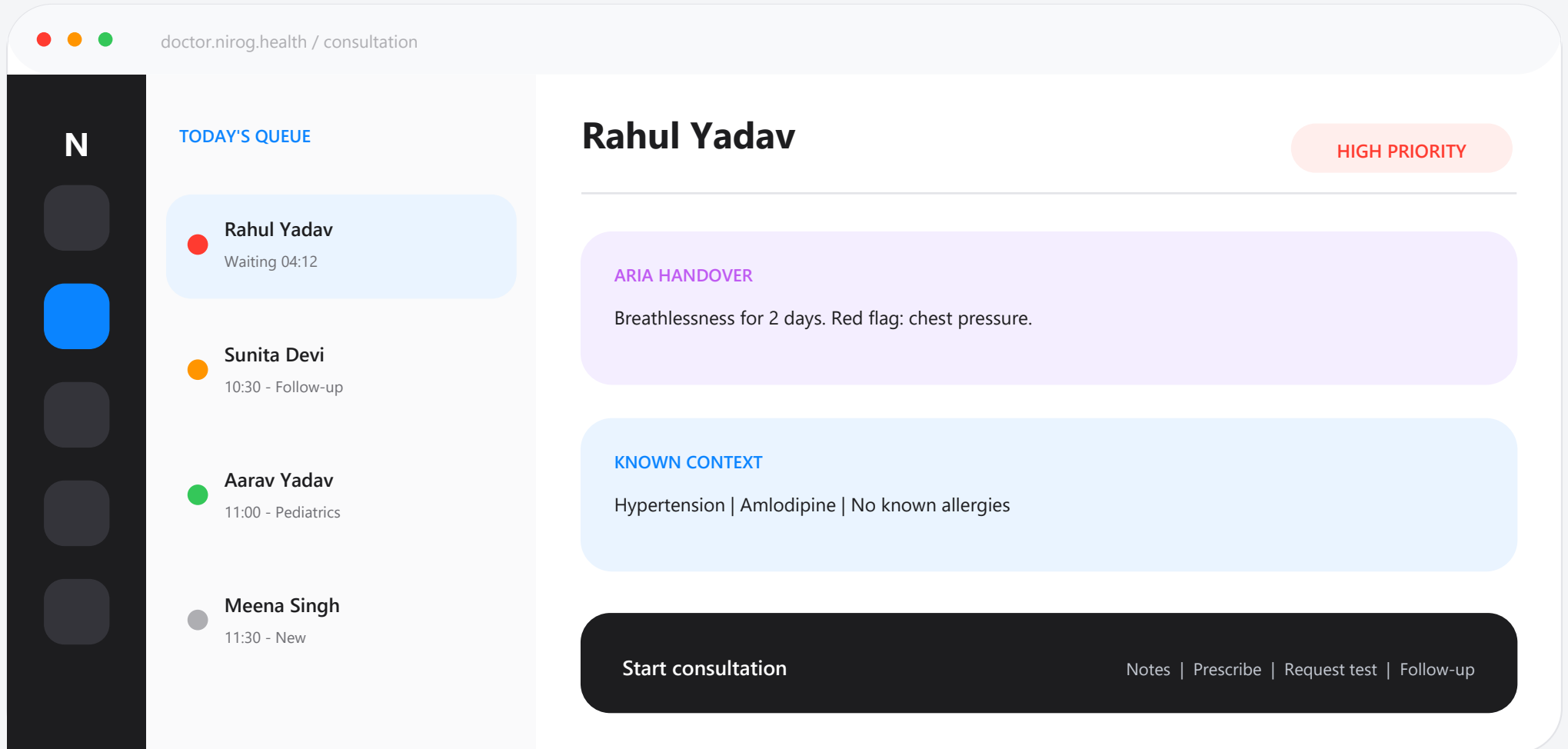
● Return safely

Store refresh credentials in device secure storage. Use biometric unlock or a passkey for convenient return access.

DOCTOR EXPERIENCE

A clinical workspace, not a stretched mobile screen

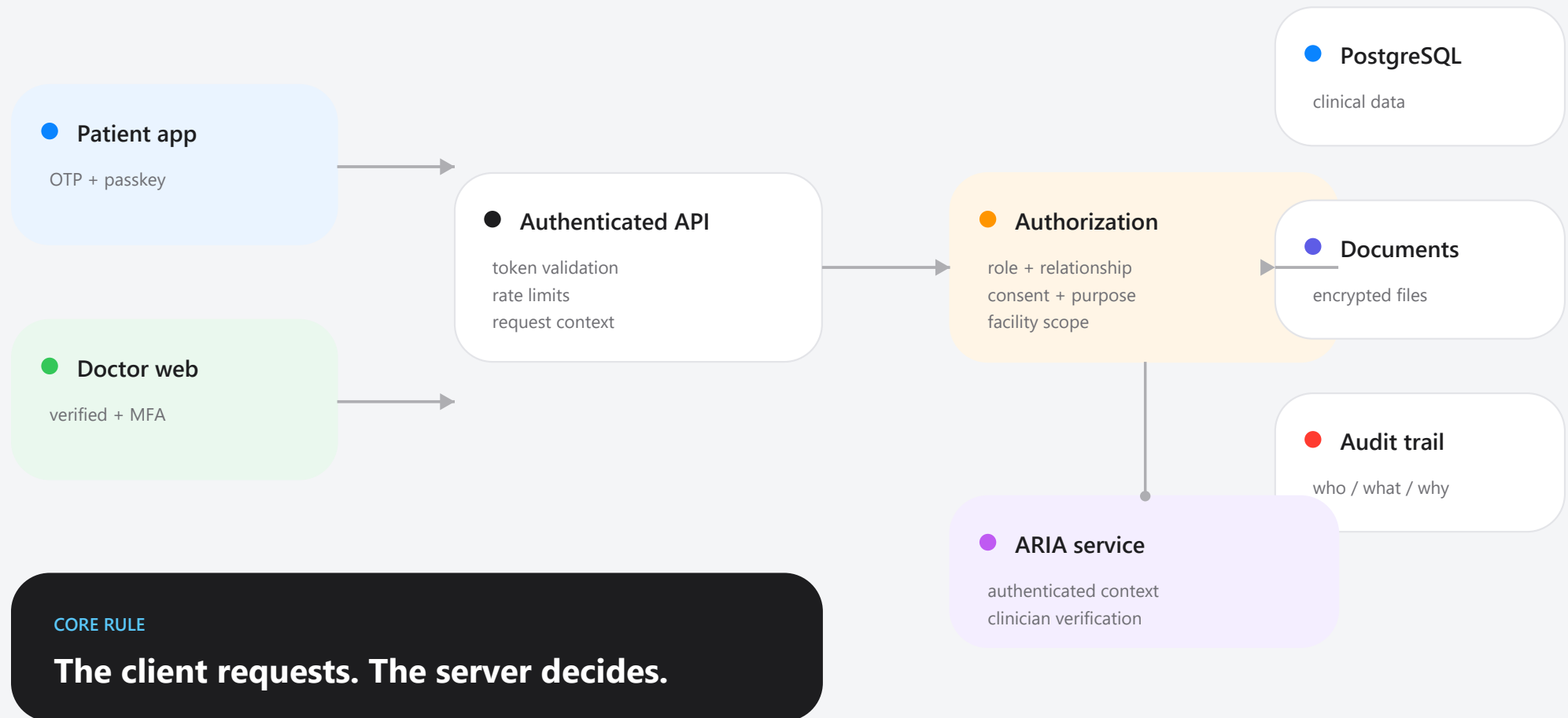
The interface should adapt its information density while keeping the consultation flow consistent.



SYSTEM DESIGN

Trust sits between every client and every record

A shared backend should enforce identity, care relationships, consent and audit before data reaches ARIA or a clinician.



NON-NEGOTIABLES

Real health data changes the engineering standard

The current demo patterns must be upgraded before clinical records, prescriptions or real consultations are enabled.

IDENTITY

- Patient tokens in secure device storage
- Doctor MFA or passkey
- Step-up auth for prescribing and exports

ACCESS

- No doctor-wide patient search
- Time-bound care relationships
- Break-glass access with a reason

DATA

- Encrypted documents and backups
- No health data in logs or analytics
- Short-lived signed file links

CLINICAL

- ARIA output is unverified
- Doctor accepts or edits before filing
- Deterministic red-flag escalation

PRIVACY

- Purpose-specific consent
- Withdrawal and deletion workflow
- Minimal push notification content

OPERATIONS

- Immutable access audit
- Credential suspension workflow
- Backup, recovery and incident plan

[DESIGN FOR DPDP + ABDM NOW](#)

Legal and clinical review remains required before launch.

DIFFERENTIATION

Design around the reality of access

Nirog can win by making care usable under bandwidth, language, literacy and family-care constraints.

1 Audio-first

Graceful downgrade from video to audio and text.

2 Regional language

Hindi first, then locally relevant languages and voice.

3 Family care

Dependents, elders and caregiver permissions built in.

4 Resumable

Uploads and consultation state survive poor networks.

5 Assisted mode

Support ASHA workers and clinic facilitators safely.

6 Closed loop

Every care plan returns as reminders and follow-up.

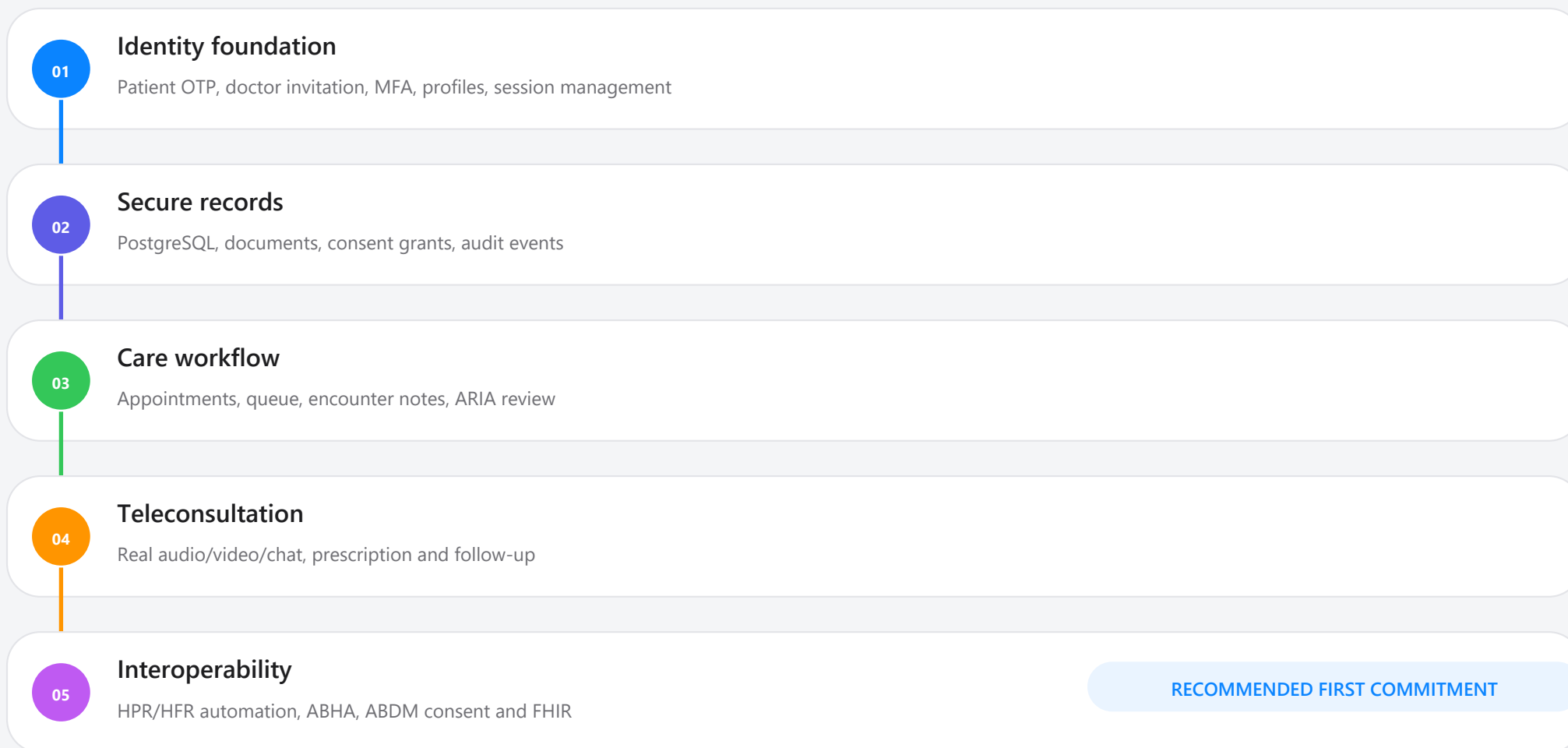
The moat is not AI alone.

It is continuity of trusted care under difficult conditions.

SEQUENCING

Build trust before transaction volume

Each release should create a usable slice of the care loop while preserving the right architecture for ABDM and scale.



FILLABLE WORKSHOP

Make the architecture decision together

Tick the agreed direction, vote on first-release priorities, then record the decision and owner.

A. DIRECTION

- ☐ Separate patient app and doctor web
- ☐ One shared authenticated API
- ☐ Managed identity provider
- ☐ Consent and audit in release one
- ☐ ABHA integration after core workflow

B. PRIORITY VOTE - PICK THREE

- ☐ Identity
- ☐ Records
- ☐ Doctor portal
- ☐ Teleconsult
- ☐ ABDM

C. TEAM RECOMMENDATION

DECISION OWNER

DECISION DATE

CONFIDENCE - 1 LOW, 5 HIGH

☐1 ☐2 ☐3 ☐4 ☐5

FILLABLE WORKSHOP

Turn the decision into owned work

Capture one owner and one concrete first milestone for each committed workstream.

WORKSTREAM	OWNER	TARGET	FIRST MILESTONE
Identity + sessions			
Consent + records			
Doctor web			
Clinical governance			

TOP RISKS / OPEN QUESTIONS

MEETING NOTES

REFERENCE

Sources behind the recommendation

Clickable links for repository context, authentication, mobile security, ABDM and India's current privacy framework.

[Nirog repository](#)[OPEN](#)

Current Expo application and product prototype

[Nirog README](#)[OPEN](#)

Five screens, ARIA and dummy-data scope

[ARIA client](#)[OPEN](#)

Current stateless clinical intake integration

[OWASP Authentication](#)[OPEN](#)

MFA, password and session guidance

[OWASP MASVS](#)[OPEN](#)

Mobile storage, authentication and privacy baseline

[ABDM overview](#)[OPEN](#)

ABHA, registries, PHR and consent architecture

[Healthcare Professionals Registry](#)[OPEN](#)

Verified clinician identity and teleconsultation

[DPDP Rules 2025](#)[OPEN](#)

Official MeitY publication and phased implementation

NOTE

This pitch is a product and architecture proposal, not legal or medical advice. Obtain specialist review before launch.